

## Early Classification of Illness

Work on the classification of illness had a 2000-year head start on the more recent efforts to classify strengths and positive outcomes. Therefore, it is easy to understand why we have better understandings of human weaknesses than we do of strengths.

The earliest attempt to define a set of virtues is contained in Confucian teachings dating to 500 BC, where Confucius systematically addressed jen (humanity or benevolence), li (observance of rituals and customs), xin (truthfulness), yi (duty or justice), and zhi (wisdom).

The ancient roots of the term clinical psychology continue to influence our thinking about the discipline long after these roots have been forgotten. Clinic derives from the Greek *klinike*, or medical practice at the sickbed, and psychology derives from the Greek *psyche*, meaning "soul" or "mind" (Webster's Seventh New Collegiate Dictionary, 1976). How little things have changed since the time of Hippocrates. Although too many of its practitioners ("clinicians") and most of the public still view clinical psychology as a kind of "medical practice" for people with "sick souls" or "sick minds."

In the 21st century, two classifications of illness have attained world-wide acceptance. First, the World Health Organization's International Classifications of Diseases (ICD) is in its 10th edition and continues to evolve. Second, the American Psychiatric Association's Diagnostic and Statistical Manual (DSM) now is in its 6th edition as the DSM-5-TR (Text Revision). The ICD is broader in scope than the DSM in that it classifies all diseases, whereas the DSM describes only the mental disorders. Currently, no classification of human strengths or positive outcomes has achieved worldwide use or acceptance. Some classifications and

measures, however, have been created, refined, and broadly disseminated in the last decade.

The final lines in this 1968 address delivered by Robert F. Kennedy at the University of Kansas point to the contents of this topic/subject, the things in life that make it worthwhile. In this regard, however, **imagine that someone offered to help you understand human beings but in doing so would teach you only about their weaknesses and pathologies.**

As far-fetched as this sounds, a similar “What is wrong with people?” question guided the thinking of most applied psychologists (clinical, counseling, school, etc.) during the last 100 years. Given the many forms of human imperfection, this question produced a flood of insights into the human “dark side.” With the founding of the **first “psychological clinic” in 1896 at the University of Pennsylvania, Lightner Witmer started the field of clinical psychology** (Reisman, 1991). **Witmer and other early clinical psychologists worked primarily with children who had learning or school problems** not with patients with “mental disorders” (Reisman, 1991; Routh, 2000). Thus, they were influenced more by psychometric theory and its emphasis on careful measurement than by psychoanalytic theory and its emphasis on psychopathology. **Following Freud’s 1909 visit to Clark University, however, psychoanalysis and its derivatives soon came to dominate both psychiatry and clinical psychology** (Korchin, 1976).

**Other developments encouraged clinical psychologists to devote their attention to psychopathology and to view people through the lens of the disease model.** First, **although clinical psychologists’ academic training took place in universities, their practitioner training primarily occurred in psychiatric hospitals and clinics** (Morrow, 1946, cited in Routh, 2000) where they worked mostly as psycho-diagnosticians under the direction of

psychiatrists. Second, after World War II (1946), the Veterans Administration joined the American Psychological Association in developing training centers and standards for clinical psychologists.

As the twenty-first century unfolds, another question, "What is right about people?" seems to attract the masses. This question is at the heart of positive psychology, which is the scientific and applied approach to uncovering people's strengths and promoting their positive functioning. (See the article "Building Human Strength," in which positive psychology pioneer Martin Seligman gives his views about the need for this new field.)

Although other subareas of psychology were not focused on human weaknesses, twentieth-century applied psychology and psychiatry typically were. For example, consider the statement attributed to Sigmund Freud that the goal of psychology should be "to replace neurotic misery with ordinary unhappiness" (cited in Simonton & Baumeister, 2005, p. 99).

Thus, the applied psychology of yesteryear was mostly about mental illness and understanding and helping the people who were living such tragedies. Positive psychology, on the other hand, offers a balance to this previous weakness-oriented approach by suggesting that we also must explore people's strengths along with their weaknesses. In advocating this focus on strengths, however, in no way do we mean to lessen the importance and pain associated with human suffering.

## 21st century classification of human strengths

*Let us imagine that one could set up a kind of scale or yardstick to measure the success of life—the satisfactoriness to the individual and the environment in their mutual attempts to adapt themselves to each other. Toward the end of such a yardstick, positive adjectives like "peaceful," "constructive," "productive," might appear, and at the other end such words as "confused," "destructive," "chaotic." These would describe the situation in general.*

*For the individual himself there might be at one end of the yardstick such terms as "healthy," "happy," "creative," and at the other end "miserable," "criminal," "delirious."*

Menninger, Mayman, & Pruyser (1963, p. 2)

**Karl Menninger**, one of the brothers who helped build the world-renowned Menninger Clinic, attempted to change the way that health care professionals viewed the diagnosis, prevention, and treatment of mental illness. As part of his mission, he encouraged clinicians and researchers to give up the old, confusing labels of sickness. Then, he called for the development of a simple diagnostic system that described the life process rather than states or conditions. Finally, he reminded us of the power of the "uplifting expressions of the life instinct" (Menninger et al., 1963, p. 357), specifically hope, faith, and love.

Over the last 50 years, psychology and psychiatry have busied themselves with the confused and miserable aspects of human nature, and, as a result of maintaining the pathology focus, health care providers have helped millions of people relieve their suffering. Unfortunately, too few professionals have engaged in the entire thought experiment described previously, and this has resulted in the unmet needs of millions more people.

We continue to add complexity to an evergrowing diagnostic system (American Psychiatric Association, 2013); we know little about the process of living; and we spend far too little time and energy making sense of the intangibles of good living—hope, faith, and love. If Menninger were alive, we think he would consider our professional expertise and knowledge lacking in utility and out of balance. Most important, he probably would ask, “What about the productive and healthy aspects of personal functioning?”

Although the missions of many positive psychologists bear similarities to Dr. Menninger’s ideas, there is a long way to go in measuring human strengths. (We subscribe to Linley and Harrington’s [2006] definition of strength as a capacity for feeling, thinking, and behaving in a way that allows optimal functioning in the pursuit of valued outcomes.)

### **Three basic qualification systems of human strengths:**

- 1. The Gallup Themes of Talent** (Buckingham & Clifton, 2001; Rath, 2007) as measured by the Clifton Strengths Finder and the Clifton Youth Strengths Explorer
- 2. The Values in Action (VIA) Classification of Strengths** (Peterson & Seligman, 2004) as measured by the adult and youth versions of the VIA Inventory of Strengths
- 3. The Search Institute’s 40 Developmental Assets** (Benson, Leffert, Scales, & Blyth, 1998) as measured by the Search Institute Profiles of Student Life: Attitudes and Behaviors

## **Deconstruction of illness ideology and inclusion of human strengths**

The short history of clinical psychology suggests that change will not come easily.

It is time to change clinical psychology's view of itself and the way it is viewed by the public. Positive psychology provides a long-overdue opportunity for making this change.

Psychology is not just the study of weakness and damage; it is also the study of strength and virtue. Treatment is not just fixing what is broken, it is nurturing what is best within ourselves.

Bringing this to the foreground is the work of the Presidential Task Force on Prevention, headed by Suzanne Bennett Johnson and Roger Weissberg. This task force will take on a number of jobs:

It will attempt to identify the "Best practices in prevention," led by Karol Kumpfer, Lizette Peterson, and Peter Muehrer;

it will explore "Creating a new profession: training in prevention and health promotion" by setting up conferences on the training of the next generation of prevention psychologists, led by Irwin Sandler, Shana Millstein, Mark Greenberg, and Norman Anderson;

it will work with Henry Tomes of APA's Public Interest Directorate in the ad campaign to prevent violence in children; it will sponsor a special issue on prevention in the 21st century for the American Psychologist, edited by Mihaly Csikszentmihalyi; and, led by Camilla Benbow,

it will ask what psychology can do to nurture highly talented children.



## **Building Human Strength:**

Psychology's **Forgotten Mission** **Martin** E. P. Seligman  
President, American Psychological Association Before World War  
II, **psychology had three missions:**

**curing** mental illness,

**making** the lives of all people more fulfilling, and

**identifying and** nurturing high talent.

After the war, two events changed the face of psychology.

**In 1946**, the Veterans Administration was created, and practicing psychologists found they could make a living treating mental illness.

**In 1947**, the National Institute of Mental Health was created, and academic psychologists discovered they could get grants for research on mental illness.

**As a result**, we have made huge advances in the understanding of and therapy for mental illness. At least 10 disorders, previously unmanageable, have understood their secrets and can now be cured or considerably relieved. Even better, millions of people have had their troubles relieved by psychologists.

**Our Neglected Missions** But the downside was that the other two fundamental missions of psychology— **making the lives of all people better and nurturing "genius"**—were all but forgotten.

We became a victimology. Human beings were seen as passive central point: Stimuli came on and elicited "responses," or external "reinforcements" weakened or strengthened "responses," or conflicts from childhood pushed the human being around.

Viewing the human being as essentially passive, psychologists treated mental illness within a theoretical framework of repairing damaged habits, damaged drives, damaged childhoods, and damaged brains.

Positive psychology comes as a new science to aims to understand, test, discover and promote the factors that allow

individuals and communities to thrive' (grow, develop, flourish) (Sheldon et al., 2000).

Positive psychology focuses on wellbeing, happiness, flow, personal strengths, wisdom, creativity, imagination and characteristics of positive groups and institutions.

Furthermore, the focus is not just on how to make individuals happy, thereby maintaining a self-centered, narcissistic approach, but on happiness and flourishing at a group level as well.

Positive psychology is not simply the focus on positive thinking and positive emotions. It's much more than that. Indeed, the area of positive psychology is focused on what makes individuals and communities flourish, rather than weaken. Flourishing is defined as a state of positive mental health; to thrive, to prosper and to fare well in endeavors free of mental illness, filled with emotional vitality and function positively in private and social realms' (Michalec et al., 2009: 391).

### **Building Strength, Resilience, and Health in Young People** **But an underlying question remains:**

How can we prevent problems like depression, substance abuse, schizophrenia, AIDS, or injury in young people who are genetically vulnerable or who live in worlds that nurture these problems? What we have learned is that pathologizing does not move us closer to the prevention of serious disorders. The major progresses in prevention have largely come from building a science focused on systematically promoting the competence of individuals.

We have discovered that there is a set of human strengths that are the most likely buffers against mental illness: courage, optimism, interpersonal skill, work ethic, hope, honesty, and determination.

Much of the task of prevention will be to create a science of human strength whose mission will be to foster these virtues in



young people. Fifty years of working in a medical model of personal weakness and on the damaged brain has left the mental health professions ill equipped to do effective prevention.

We need massive research on human strength and virtue. We need practitioners to recognize that much of the best work they do is amplifying the strengths rather than repairing their patients' weaknesses. We need psychologists who work with families, schools, religious communities, and corporations to emphasize their primary role of fostering strength.

**The major psychological theories** have changed to support a new science of strength and resilience. Individuals—even children—are now seen as decision makers, with choices, preferences, and the possibility of becoming masterful, efficacious, or, in hostile/harmful circumstances, helpless and hopeless.

Such science and practice will prevent many of the major emotional disorders. It will also have two major effects. Given all we are learning about the effects of behavior and of mental well-being on the body, it will make our clients physically healthier.

It will also **re-orient** psychology to its two neglected missions, making normal people stronger and more productive as well as making high human potential actual. Source: Seligman, M., "Building human strength: Psychology's forgotten mission," in APA Monitor, 28(1): January 1998, p. 2. Copyright 1998 by the American Psychological Association. Reproduced with permission.

## **NEED FOR A SCIENCE OF HUMAN STRENGTHS AND VIRTUES**

While the formal discipline of positive psychology has only existed since 2000, the concepts that form the basis of it have been the subject of empirical study since at least the 1980s, and present in religious and philosophical discourse for thousands of years. It has been influenced by humanistic as well as psychodynamic approaches to treatment. Predating the use of the term “positive psychology”, researchers within the field of psychology had been focusing on topics that would now be included under this new denomination

The term positive psychology dates back at least to 1954, when Maslow’s first edition of *Motivation and Personality* was published with a final chapter titled “Toward a Positive Psychology.” In the second edition published in 1970, he removed that chapter, saying in the preface that “a positive psychology is at least available today though not very widely.”

There have been indications that psychologists since the 1950s have been increasingly focused on the promotion of mental health rather than merely treating mental illness.

Positive psychology grew as an important field of study within psychology in 1998 when Martin Seligman chose it as the theme for his term as president of the American Psychological Association. In the first sentence of his book *Authentic Happiness*, Seligman claimed: —for the last half century psychology has been consumed with a single topic only “mental illness,” expanding on Maslow’s comments. He urged psychologists to continue the earlier missions of psychology of nurturing talent and improving normal life.

Several humanistic psychologists, most notably Abraham Maslow, Carl Rogers, and Erich Fromm, developed theories and practices

pertaining to human happiness and flourishing. More recently, positive psychologists have found empirical support for the humanistic theories of flourishing. In addition, positive psychology has moved ahead in a variety of new directions.

In 1984, Diener published his tripartite model of subjective well-being, positing “three distinct but often related components of wellbeing: frequent positive affect, infrequent negative affect, and cognitive evaluations such as life satisfaction.”

